

# Shattering Glass Ceilings

## Three Black Women Surgeons Ascend to Chairs of Departments of Surgery

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Diversification at the highest level of academic surgical leadership is critical in creating a diverse, equitable, and inclusive environment; an environment more reflective of the population whom we as surgeons serve and care for. Until the year 2021, an African American female surgeon had never ascended to the role of Department of Surgery Chair. However, by 2021, not 1 but 3 African American women became Chairs of Surgery: Karen Gibbs, MD, KMarie King, MD, and Andrea Hayes, MD (Fig. 1). To highlight and celebrate these historical appointments, we interviewed these extraordinary women who shared their unique views on ascending to leadership. To capture the true impact of these historical appointments, we also interviewed 5 African American female surgeons (Carla Pugh, MD, PhD, Lola Fayanju, MD, MA, MPH, Callisia Clarke, MD, MS, Yewande Alimi, MD, MPH, and Allison Martin, MD, MPH) and 1 African American medical student (Jaina Lane, BA) as a group and have shared their thoughtful and insightful commentary below (Fig. 2). Interviews were conducted virtually with questions asked by the authors to the group in bold and transcribed responses described below. Participant responses to the group interview questions are reported with anonymity. Consent was obtained from all participants.

### KAREN E. GIBBS, MD, FACS, FASMBS, FABQAURP, DABOM

Karen E. Gibbs, MD, FACS, FASMBS, FABQAURP, DABOM is the Chair of Surgery at Yale New Haven Health—Bridgeport Hospital. Appointed to the position in September of 2021, Dr Gibbs specializes in laparoscopic, bariatric, foregut, and endocrine procedures. Her clinical practice and research focus on advancing the complex and integrated understanding of weight loss for the management and reversal of obesity-related disease and disorders of aging, including diabetes and sleep apnea.

Dr Gibbs received a bachelor's degree from Boston University, attended Tufts University Medical School, and completed her General Surgery Residency and Fellowship in

Bariatric and Minimally Invasive Surgery at Montefiore Medical Center, Albert Einstein College of Medicine.

After starting her practice at Montefiore Medical Center, as Faculty for the Albert Einstein College of Medicine, Dr Gibbs was asked to join the Bariatric Surgery team and naturally moved into bariatric surgery leadership. Subsequently, she moved to Staten Island University Hospital, Northwell Health and assumed multiple leadership roles. She served as Associate Chair for the Department of Surgery; Chief of General, MIS & Bariatric Surgery; Director of the General Surgery Residency Program and the Advanced GI MIS Fellowship; and Director of Performance Improvement & Quality and Associate Medical Director for Quality at Staten Island University Hospital.

Dr Gibbs was chosen to lead the surgery Quality program in part because one of her credos is fairness in the work environment, which was well known by hospital leadership. As Quality lead she had the opportunity to work with all surgical specialties and recognize the similarities, and differences, surgeons face in their unique practices. An essential component of her role involved interaction with nursing staff, who were key in facilitating patient care quality improvement. She had the opportunity, and willingness, to step up and ask for resources to improve patient care for all surgical specialties and was energized by the opportunity to work with multiple divisions and departments, “moving the needle,” improving both patient care and the work environment. Dr Gibbs was initially taken by surprise when her relocating Chair proposed her for the Interim Chair role. She had previously held leadership positions within the hospital and her performance in those roles was applauded. In fact, the hospital President suggested she seek a Chair position due to her strong leadership and interpersonal skills and an ally encouraged her to apply for the Yale New Haven Health—Bridgeport Hospital Chair position. Once she spoke to the hospital and department leadership, she realized that there was a real connection and that the position was a fit for her personally and professionally. Although she had never had a personal goal of becoming a Chair, her ongoing focus was always to be “a really good surgeon, taking great care of patients and providing high quality service to the community.” That focus led to additional roles and opportunities, and she strongly recommends that young physicians “master their craft,” establish their research focus and “create their brand” to excel in leadership roles. It is important to “apply yourself and enjoy the journey.”

When asked her thoughts on why it took until 2021 for Black women to become surgery Chairs, Dr Gibbs felt there may have been an element of societal complacency considering the progress that had been made in advancing the number of women in surgery. However, there is now a global movement and a collective awakening which will hopefully continue to carry Black women forward into leadership roles. She notes that Yale has a strong history of diverse leadership, so that trainees have role models in leadership positions. She emphasized that holistic interviewing and intentional hiring are essential to create

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Henry and Sally Schaffer Chair of Surgery  
Chief of Surgery  
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**Andrea A. Hayes, MD, FACS, FAAP**  
Professor and Chair of Surgery  
Howard University

**FIGURE 1.** Newly appointed chairs of surgery.

a diverse surgical workforce. Institutional leadership should prioritize diversity, and diverse team members must be engaged, hold meaningful positions, and be intimately involved in decision making and creating policies. To avoid the “minority tax,” it helps to have “a community of individuals” that can share the weight of the mission and have recognition, value, and reward assigned to their efforts. Allyship is also essential to move DEI efforts forward. Dr Gibbs recognizes that pay equity and parity are issues in surgery and voiced that the key is consistency and a transparent process involving review of current standards, including percentiles, academic rank, and research contributions.

Like any new Chair, Dr Gibbs is currently learning the culture of her department and becoming familiar with the breadth and complexity of the organization. She is impressed with the surgical expertise of her team and has an exemplary administrative and finance team that is orienting her to open business plans and departmental/institutional strategic initiatives. Going forward, her goals include creating an environment that fosters clinical excellence, intellectual curiosity, and professional growth. Creating such a milieu will engage and bring joy to the surgical team members, and allow them to “take care of each other, as they care for their patients.”

### **KMARIE KING MD, MS, MBA, FACS**

On September 1, 2021, KMarie King, MD, MS, MBA, FACS, Professor of Surgery, the Henry and Sally Schaffer Chair Department of Surgery, and Chief of Surgery for Albany Medical Center, became the first African American woman appointed Chair of an academic health care center in the United States.

Dr King received her Bachelor of Science degree in biology from St Joseph's College in Brooklyn, NY. She went on to receive her medical degree from Washington University School of Medicine and was the first Black female to match into the General Surgery Residency training at the University of Pittsburgh in 1998. Dr King then moved to Rochester, MN to complete her GI fellowship training in Hepatobiliary & Pancreas Surgery at Mayo Clinic. She was the first Mayo Hepatobiliary & Pancreas Surgery fellow hired to stay on as faculty. While at Mayo, she was promoted to Associate Professor of Surgery and received a Master of Science in Biomedical Sciences. She remained at Mayo Clinic, as the only Black surgeon on faculty, for 11 years.

Dr King is a prolific surgeon scientist with over 95 peer-reviewed publications and book chapters. She was awarded many institutional research grants and a K Award in 2007 from the National Institutes of Health for her work in inflammation-related

genes as risk factors for pancreatic cancer. Dr King has held 3 journal editorial positions and is currently a scientific reviewer for several scientific journals. She is a leader in academic surgery with numerous prior leadership positions in the Association for Academic Surgery, Society of Gastrointestinal and Endoscopic Surgeons, the Society for Surgery of the Alimentary Tract, and the Society of Black Academic Surgeons (SBAS).

While describing her experience as a young hepatobiliary and pancreas surgeon, flourishing, and excelling as a surgeon, researcher, educator, and leader, she knew that she could not “do it all” and she would have to decide on 1 or 2 areas of focus. She had the opportunity to participate in the American College of Surgeons Leadership Program in Health Policy and Management at Brandeis University as a Society of the Alimentary Tract scholarship recipient in 2015. After successfully completing this program, she was invited to apply for the inaugural MBA program at Brandeis University. She subsequently accepted and completed the program in 2017. In speaking about pursuing a MBA, Dr King says that she values her MBA because it has given her a new language to help persuade and influence those in power who oftentimes have a nonmedical background. After this experience, Dr King decided that Quality would be her area of focus.

Dr King was recruited to Morehouse School of Medicine as the Director of Quality for the Department of Surgery, and Section Chief for Liver, Pancreas & Foregut Surgery. She would later become Chief of Surgery at Grady Memorial Health System in Atlanta, GA and the inaugural Medical Director of Surgical Quality and Co-Champion for NSQIP. It was through her experience at Grady where she grew, flourished as a leader, and was “seen.”

Dr King spoke extensively on the importance of strong allyship and describes an exceptionally strong support system from her husband, family, and friends. While her journey has had ups and downs, twists and turns, through it all she has remained focused on being a technically skilled surgeon, an educator, a researcher, and an exemplary leader.

When asked if she had always wanted to become a Chair of Surgery, she said “As a Black woman, I didn't know that I could have that dream.” In responding to the question of why it has taken to 2021 for a Black woman to ascend to the role of Chair, Dr King says that many women, including Black women, may not believe that they are ready to become Chair and others may share that belief. Now seated in the role she knows she was “beyond ready and able.”

Dr King acknowledged that she, Dr Gibbs, and Dr Hayes are “breaking glass ceilings.” However, she also noted the “glass



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**FIGURE 2.** Group interview participants.

cliff" phenomenon and the challenge maintaining influence as a woman and as Black woman leader. She recalls asking the Dean during her interview "Are you prepared to have a Black woman Chair? There will be a lot of people trying to work around me; how are you going to support me when that happens?" His response

was very direct; "If they cannot deal with you, then they can leave." In accepting the position of Chair, Dr King explained that she felt aligned with Albany Medical Center, where she felt a deep connection to the people. She strives to be a compassionate leader, to have empathy, and to lead with integrity.

In speaking on advancing DEI, Dr King says that she has revamped the faculty recruitment process at Albany. She now requires a diverse slate of candidates for consideration. She will then select the candidates to be interviewed based on the vision she has for the department. Dr King speaks on the importance of having different voices at the table, including the C-Suite, to help reduce bias and ensure that there is equity in health care delivery.

Dr King's advice for young surgeons is to first, find a community, such as SBAS, to become your support system. Societies such as SBAS provide access to Division Chiefs, Department Chairs, Deans, and leaders in surgery who are there to mentor you, and who take that role very seriously. Second, seek out an environment that feeds and nurtures you. If you are in an environment where you are not thriving and you are not in full alignment with the culture of the organization, you will not be successful. You must know yourself and know yourself well to make those choices. Third, have your organization invest in a professional coach for you. "If we want to perform at our highest and best selves, we need to have coaches." Finally, it is not always about becoming a Chair. Sometimes it is just about doing great work where you are and to focus on the work. Dr King lives by the credos "The patient comes first...be kind...and have integrity."

### ANDREA A. HAYES, MD, FACS, FAAP

On January 5, 2022, Andrea A. Hayes, MD FACS FAAP became Professor and Chair of Surgery at Howard University in Washington, DC. Dr Hayes completed both her undergraduate and graduate medical studies at Dartmouth College in Hanover, New Hampshire. Subsequently, she completed her General Surgery Residency at the University of California Davis in East Bay, being the first Black woman to graduate from that residency. Dr Claude Organ, Jr, the Chair of Surgery, became a lifelong mentor and ally. During residency, she was a Molecular Biology Fellow at the University of California in San Francisco. Following General Surgery Residency, Dr Hayes was a Pediatric Surgery Oncology Fellow at St Jude's Children's Research Hospital and a Melanoma and Sarcoma Fellow at the University of Texas MD Anderson Cancer Center in Houston, Texas. Her Pediatric Surgery Fellowship was completed in 2002 at the Hospital for Sick Children in Toronto, Ontario. She was the first Black woman accredited by the American Board of Surgery, Pediatric Surgery and became the nation's first Black female Pediatric Surgeon in 2004.

Dr Hayes has served in many leadership roles, including as Program Director for the General Surgery Residency and the Pediatric Surgery Oncology fellowship at the University of Texas MD Anderson Cancer Center. She also led as Section Chairs of Pediatric Surgery and Pediatric Surgery Oncology at MD Anderson. Through 2021, she served as Division Chief of Pediatric Surgery at the University of North Carolina (UNC) Hospital in Chapel Hill, North Carolina, and as Surgeon-in-Chief of the North Carolina Children's Hospital. She served as a Director of the Pediatric Surgery Board of the American Board of Surgery and as President of the SBAS. In addition, Dr Hayes is a member of the National Cancer Advisory Board CCDI Steering Committee. She served as a member of the American College of Surgeons (ACS) Board of Governors and in recognition of her expertise and accomplishments was elected to be a Regent of the ACS in October 2021.

Dr Hayes is a formidable surgeon scientist with 142 peer-reviewed publications and the Principal Investigator role for several clinical trials. In her 2021 ACS Clinical Congress Olga

Jonasson lecture, Dr Hayes spoke of "Grit in Spite of Adversity." She defines grit as "persistence plus perseverance," and has relied on her passion, grit, and faith to achieve her goals. Dr Hayes is an international expert in pediatric oncology and, in 2005, was the first surgeon to successfully perform cytoreductive surgery and Hyperthermic Intraperitoneal Chemotherapy in a child. Since then, she has become the expert on treating Desmoplastic Small Round Cell Tumor in children and has developed a novel orthotopic xenograft model of Desmoplastic Small Round Cell Tumor in NOD SCID gamma mice for basic science research in her well-funded laboratory.

While assuming a Surgical Chair role had not been on Dr Hayes' radar, she found that she enjoyed the opportunities afforded her in the role of Section Chief of Pediatric Surgery. Her role as Surgeon-in-Chief at UNC allowed her to change the institution and the environment, enhancing patient care. She enjoyed engaging with the other Chairs at UNC and encouraging growth in her faculty. She sees her role at Howard to be even more meaningful, considering the incredible legacy of the Chairs before her and the opportunity to be a woman Chair at a Historically Black College & University (HBCU).

Dr Hayes is a strong supporter of mentorship, and is appreciative of her mentors, particularly Dr Diana Farmer. She noted that Dr Farmer reached out to her, which was critical. Dr Hayes feels that lack of mentorship may be one of the reasons that Black women surgeons have not previously risen to Chair positions, and that it is incumbent upon surgeons to reach out and mentor younger surgeons who have the potential to become leaders in academic surgery. Having female surgical role models is also essential to the success of medical students and surgical residents.

Overcoming bias is a challenge as a Black woman surgeon. Dr Hayes noted that a favorite quote is from Dr Charles Drew: "Excellence of performance will transcend artificial barriers created by man." However, she has noted that women surgeons in her position must work harder, and jump over more hurdles, to overcome preconceived ideas about their performance. When confronted with bias from colleagues and coworkers, she emphasized the importance of allyship and education. Intentionality in hiring of leaders from URiM groups is also essential, and she has seen the positive effects of that at Howard University.

Dr Hayes plans on leading the Surgery Department at Howard University and continue building the Howard University Cancer Center as its Associate Director. She is passionate about health care policy as an avenue to enhance patient care and is sharpening her skills in her role as a National Cancer Advisory Board CCDI Steering Committee member.

Dr Hayes' advice for young surgeons is to appreciate that they are already leaders in the operating room and of their teams. Being a "good leader" means being kind, incorporating other's feelings into decisions you make, being sensitive to the feelings of your team, supporting your team members' individual goals and "leading with excellence." She also recommends surgeons show up prepared, contribute to the conversation, be respectful of others, and be "present"; "be the best resident or faculty surgeon that you can be."

### Group Interview

Five Black women surgeons and a medical student were interviewed virtually as a group to gather their reflections on the ascension of Black women into leadership roles in surgery. Five questions followed by responses from individual members of the group are found below.

**Question 1: This year was marked by historical firsts and shattering of glass ceilings with the appointment of Dr Patricia Turner as Executive Director of the College, Dr King, Dr Hayes, and Dr Gibbs to Chairs of Surgery. What are your thoughts on why it took this long; what were some of the barriers; what ultimately contributed to these historical appointments; and how do we keep progressing?**

*Response:* Black women have always been last. In our culture, there is a caste system based on gender and race especially when it comes to leadership. There is a social order rooted in intersectionality. Historical data confirms that most organizations, societies, and institutions diversify first with a white woman or Black male but rarely with a Black woman.

*Response:* There is a subjugation of African American women, and we are often considered as an afterthought. When those in power are looking for leaders, they tend not to look for or consider Black women.

*Response:* I suspect part of the reason we have seen the patterns we've seen is that for a lot of organizations, the initial step to diversify is frankly a box-ticking mechanism, which is typically checked by people who are diverse but perceived to "belong in the club," so they get in. The question is, does that box ticking translate into real cultural change and are the people who check that box part of making that change happen? The role of the first diversifier can set the trend and if they have people around them who are also aligned with a trend towards authentic diversification, who recognize that excellence is part and parcel of that, then I think this will lead to accelerated inclusion of excellent Black women. If it is a box-ticking organization that maintains a box-ticking approach, and the people who check that box are not truly committed to bringing other people into the fold, then we will not see change happen.

*Response:* I would say over the last 3 or 4 years, that 0 is such a powerful number. The fact that we [Black women] had been absent from this space for so long, and it seemed like everybody was comfortable with it because it was not getting talked about. We had more than 20 female chairs of surgery before 2021 and with each woman chair in the last two to three years it was a little painful to see the celebration of gender only, without the acknowledgement that we needed to do something different. Many of us felt this way but did not know where to go with those feelings. We are excited every time there is a woman who is promoted to a position of leadership in surgery, but our hope has been over the last few decades that it would not have taken this long for Black women to be granted these same opportunities. I honestly do not know if 2020 had not happened, if 2021 would have. There is some real work to be done to try to figure out why Black women are either not seen as leaders or not seen as leaders worthy of sponsorship, in the same way that other demographic groups are.

*Response:* It is a very inspiring moment which I hope is not a passing one. I hope that the energy that catalyzed this elevation of Black women to a position they have earned and rightfully deserve isn't something that just peters out and gets lost such that people no longer feel driven to support Black women in stepping into the forefront of surgical leadership positions in this country.

*Response:* I don't know that Black women would have been offered these positions, and become Chairs, without the collective trauma of 2020 that has shone a light on our country's failure to have the kind of workforce and medical system that allowed people of color to be heard and seen when they were disproportionately affected by COVID-19. People have realized it is at our peril not to be equipped to treat a diverse population.

But this timing means that Black women are now ascending to positions of leadership in a particularly difficult moment where the likelihood of failure is very high. They are not just breaking glass ceilings; they are very much on glass cliffs. When I think about how challenging it is to lead right now in health care with the morale of staff being so low, we are hemorrhaging people at all levels of health care ... tempers and fuses are short, and I'm really struck by the fact that here we are celebrating leadership opportunities made available to Black women at a time when it is incredibly hard to be a leader.

*Response:* From a trainee's perspective, we are excited to see Black women in these surgical leadership positions and I think that representation is inspiring students who may not have considered surgery. We now see so many people of color matching into the surgical fields, especially Black women. Sustainability is really important because as we continue our training, we are going to be in your positions someday soon, and so my hope is that the same desire to have diverse representation and to have people from all backgrounds in the surgical workforce continues because the interest is not going to go away.

**Question 2: It has been said that you can't be what you can't see so what do you think is going to happen now that these charismatic, strong women are in these roles?**

*Response:* I'm hopeful, and I think that we are at a pivotal point. As a profession, I think there are a lot of Black women who are prepared for the moment and are going to benefit from meaningful mentorship and sponsorship. We have always been a very resilient group, and we recognize that it really does take a supportive tribe for anyone person to achieve these positions of leadership. I can speak for my generation—we try to make sure that we are moving as a unit, we are not making one step for one person, instead we're supporting a critical mass of people moving in the same direction. It is essential to have not just one person promoted through the system, because then there's no one in the pipeline for leadership. We are very focused on trying to attain that critical mass at the Assistant and Associate Professor levels. Influential women like Dr Pugh, Dr Hayes, Dr King, and Dr Turner have done the work to ensure that the pipeline is there. There is a plethora of talented Black women who are ready, able, and willing to lead. It has taken a while to get here, but I think having these 3 women at the forefront leading is crucial. They all understand the assignment, and they are making sure that they are not going to be just the first three.

*Response:* Hopefully, what will also be seen is the work that has been done, and that continues to be done, behind the scenes. Often, people just see the tweets about the successes or the publications, but they don't see the work behind the scenes, or the digs that you have to deal with all day from residents who undermine you, or from nurses who do not want to partner with you. While it is easier than it was, it is still often so much to bear. To see the work that Dr Pugh, Dr Turner, Dr Hayes, and Dr King and others who have quieter profiles, but who are doing the work in the shadows, to know that they were doing that work when it was even harder...I'm exhausted just thinking about it. I cannot emphasize the importance of peer mentorship and I think that honestly that has been modeled by their generation. They have been together, and they have lifted each other up, celebrating each other's achievements and successes. Whether it's Dr Ala Stanford's being a CNN hero or Dr Pugh's appointment as Vice Chair of something that is *not* Vice Chair of diversity is important. It is so rare to have a Black woman lead Sections, Divisions, and be Vice Chairs for something that is not diversity,



equity, and inclusion. We are more than DEI. Thus, having seen the success of that generation, they have provided a model that we are all committed to perpetuating.

*Response:* My dad has been the person who has always said to me I can do anything in the world, and that was something that he instilled in me from a very early age. I will say, though, there are plenty of thoughts in my mind that make me feel like that's not something that I can attain, but I think that seeing these three women in this position now, at this time, helps me feel like it's possible. It's really hard for us to see it in ourselves that we can attain it, but I'm thankful to have had people around me who do.

**Question 3: What do you think these women are going to do for the pipeline?**

*Response:* We still have a long way to go in terms of representation and ethnic and racial diversity in General Surgery. However, we are losing so many trainees in that pipeline. We are losing 20 to 25 Black residents a year, either being dismissed or being withdrawn, which we all know, is the same thing. One of the immediate hopes that I have is that we can try to address just how leaky the pipeline is and just how vulnerable Black residents and particularly Black women residents are in the training space. It's a minefield—You are too confident, or you don't smile enough or you're angry or you don't take feedback well. We have all sat there and listened to the many different adjectives that are used to describe Black women. The stereotype threat is so real, so strong, and so dangerous. My hope is that the way in which Black women in the pipeline are perceived will change.

*Response:* The bigotry of low expectations is a term that describes this phenomenon. As you are going through training, if things are not going great, is anyone telling you or do you just suddenly get dismissed? Is anyone giving you the feedback that they would give their buddy's son who they admitted to that residency program? Probably not because they are afraid that they do not want to be seen as a racist or as a misogynist. However, by not giving the feedback to be as excellent as you can be, they are being both racist and a misogynist as well as being a bad teacher. This is why everyone needs to be educated because there are so many ways in which to fail Black women. You can fail us by not keeping us, but you don't keep us if you don't train us, and many do not know how to train people in a way that really helps them. In looking at the heritability of medical privilege, many women surgeons are themselves the children and grandchildren of doctors and surgeons where they for example, have spent years practicing their father's or grandfather's "vascular anastomoses." Not everyone has had this benefit. There are so many ways that we can potentially fail Black woman in those committee meetings and in their evaluations. Dismissals without apparent provocation leave people reeling and shattered for years. There is psychological harm and PTSD that comes from being dismissed.

*Response:* Regarding these three females who are now Chairs, we are holding our breath. It's that part in the movie when you are there, and they are getting ready to open the door and you don't know what's going to happen. Come back in 10 years when there's more and they have kept their jobs. We're holding our breath because it's different. It's a double tax.

**Question 4: How do we protect Black women from the minority and gender tax?**

*Response:* This is a new conversation in that there has never been a platform that has been safe to have that

conversation. There is not much research about this. The irony is that people who will work to sponsor us and promote us have no idea what it is like to walk in the halls of the hospital and experience what we experience. They have the power to put policies in place but do not have a clear idea what the solutions are. Again, that platform has not been built, and we have not made that shift yet, but there are a few of us who are ready to have that conversation because we have agency.

**Question 5: Has anyone in your career ever encouraged you to be a Chair, a Dean, a CMO, or COO and if you wanted to pursue those roles, do you feel confident in knowing what the road looks like to achieve those positions and would you feel supported?**

*Response:* I'm thinking about impact and about legacy. I'll take any job I need to take to make the pipeline look brilliant because that's my goal. I don't know what that step is, and it may not be Chair. It may need to be something else because I am fired up about the lack of understanding of the daily experience. I have the research skills and the agency and the presence in the room with some of the top white male and female leaders in surgery to have that conversation. There's a whole lot of work to be done, so I am weighing the options, but yes, the mentorship only came after I reached a certain point of visibility in my career.

*Response:* It was never really in my path or platform because I couldn't see it. Recently, allies like Lola Fayanju, Colin Martin, and KMarie King have said, "I think you could do this if you really wanted to." Amongst ourselves we are really trying to make sure that we feel that we can be anything. We don't all have to be Chairs; we just need to make a meaningful impact in some way. I definitely had it said to me once by Dr Jeffrey Matthews when I was a 4th year medical student interviewing for a General Surgery Residency position, but after he left the institution one month into my internship, it was never said to me again and thus, it did not stick, and I had no idea how to navigate that path until recently.

In summary, responses from the group interview could be grouped into the following broad themes:

- (1) Black women surgeons are not in the pool of candidates routinely considered for leadership roles.
- (2) While institutional DEI initiatives may be promoted, a true belief in the benefit of a diverse workforce for quality patient care does not always exist.
- (3) The racial events of 2020 had a significant effect changing the culture of surgery.
- (4) Having gender and race-concordant role models in leadership roles is pivotal.
- (5) A significant amount of foundational work has been done by Black women surgeons to promote their success as a group.
- (6) Day-to-day challenges faced by Black women surgeons, including microaggressions and gaslighting, are not recognized by current surgical leadership and profoundly affect the surgical workforce pipeline.

Thus, to achieve diversification at the highest level of academic surgical leadership inclusive of Black women, the global DEI movement and collective awakening as described by Dr Gibbs will hopefully continue to carry Black women forward into leadership roles where they will be supported and valued within an environment free from bias.

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